CONSULT FOR AFRICA
Build. Transform. Grow.

THE GROWTH AND MARKETING BOOK

Growing a firm that is bought on trust

How Consult for Africa finds, wins, grows and keeps clients. The evidence on how organisations buy advice, the state of the market we sell into, the growth system, the play for each kind of buyer, and the weekly rhythm that runs it. A course for everyone who brings work into the firm.

Consult for Africa
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Owner: the growth team · Revised every quarter

BEFORE YOU START

How to use this book

> This is the single reference for how Consult for Africa grows. When a plan, a pitch or a campaign contradicts it, the book wins until the book is changed, and when the book is wrong, change the book.

Who it is for

Everyone who brings work into the firm or keeps it here: the partners and directors who lead engagements, the business development lead, the marketing team, the Office of the Founding Partner who keep the record, and every consultant on a client site, because most of our next work is noticed by the person already in the building. It assumes no prior context.

How it relates to the other documents

The **Marketing Handbook** holds the brands, the voice and the anonymised case study library. The **Marketing Operating Brief** sets the agency's service levels and KPIs. The **Growth Pack** is the business development lead's briefing on each client and the live pipeline. This book sits above all three and explains the system they belong to, so it points to them rather than repeating them.

How it is built

Five parts and 28 modules. Each module opens with what you will be able to do after reading it, teaches one idea, shows what is done and what is pending, and ends with questions to check yourself. Read Parts I and II in full. Read the modules in Part III and IV that match your work. Keep Part V open.

The one idea

Clients buy advice from people they trust, and trust reaches them through other people. The firm grows when more people can vouch for us, when they can find and understand us once they have heard our name, and when each engagement leads to the next one.

Today nearly all of that trust runs through one person, Dr Debo Odulana, and his relationships from Doctoora, Evercare, Cedarcrest and the boards he sits on. That is why our win rate is high and our pipeline is thin and unforecastable. The purpose of this book is to turn one man's network into a firm's reputation without losing what makes the network work.

When you are unsure what to prioritise, ask which choice puts us in front of more people who could vouch for us, or turns more of today's clients into tomorrow's.

Status language used throughout

WORD	MEANING
Live	Running and usable today. You can point a client or a prospect at it.
Built, unused	Exists in the platform or the repository but nobody is running it. Do not promote it until it has an owner.
Pending	Agreed, not yet built or not yet done.
Founder decision	Blocked on a call only Debo can make. Module 28 lists every one.

Status is as of 13 October 2026. Figures carry a small number that points to the source list in Appendix A. A figure without a number is our own data.

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PART I

Foundations

What we sell, the market we sell it into, how our buyers actually decide, who they are, what we say to them, and the lines we never cross. Everything else in the book rests on these six modules.

1. What we sell and how it is bought
2. The market we grow in
3. How organisations buy advice
4. Who we serve
5. Positioning and the message house
6. The rules we never break

MODULE 1

What we sell and how it is bought

> After this module you can name the five things clients actually sign, explain the five-stage progression, and say why the audit is the commercial engine of the firm.

Consult for Africa is a healthcare management firm that has run hospitals. We are hired to find out what is really wrong, decide what to do about it, and then do it, which often means running the thing afterwards.

The website publishes seven service lines (clinical governance, strategy and growth, fractional leadership, digital health, health systems strengthening, diaspora expertise and the healthcare marketing agency) and an eighth page for claims recovery went live on 7 October. Clients do not buy from that menu. They sign one of five things, and the menu is how we describe the expertise behind them.

WHAT CLIENTS SIGN	WHAT IT IS	WHERE WE HAVE DONE IT
Organisational audit	A structured diagnostic across ten domains, ending in a maturity assessment and a set of decisions for the board	Osteon, Arabella, Haven phase two, Dennis Ashley
Turnaround and growth mandate	A target, a set of levers and delivery against it	Haven phase three, House of Refuge
Board governance	Boards, committees, delegation of authority, director training	Havana Specialist Hospital
Venture structuring	Holding companies, joint ventures, cap tables, shareholder terms, alongside the client's counsel	The Kemi Balogun group, the Medbury aesthetics vehicle
Build and operate	We design and set up the facility or the venture, then manage it under an agreement	The Medbury division, Lyfe Place Abuja, ilé

The progression, and why the audit matters most

Most of our engagements move through the same five stages, though not every one goes the whole way. Each stage has its own document and its own conversation, and the commercial point is that each stage sells the next.



Alongside every stage: leadership assessment, recruitment, workforce data, claims recovery and marketing, which the products and the HR prac

The audit is highlighted because it is the stage a stranger will buy, and it reliably finds something larger than its own brief.

Figure 1. The engagement ladder. Each stage is priced to stand alone and designed to show the client the next one.

The audit is the commercial engine because it is small enough for a hospital owner to say yes to without a board meeting, and because it almost always finds that the presenting problem is not the real one. Haven came to us about stock and the binding constraint was culture and incentives. A surgeon who believed his postcode was the problem turned out to have no referral pipeline. When the audit is done well, the client asks us what to do next, and that is the cheapest sale this firm ever makes.

DONE

- Five things clients sign, each with at least one live or completed engagement
- Rate card live in code and visible to directors at /finance/rate-card
- Two clients have moved from audit to a larger mandate (Haven, Medbury)
- One operate mandate signed with a group (the Medbury division, September 2026)

PENDING

- No standard audit offer page a stranger could read and buy from
- No case study route on the website, so none of the progression is visible
- Agency has no rate card (founder decision)
- Recruitment fee band recorded in two places at two levels; reconcile to the rate card

CHECK YOURSELF

1. A hospital owner says "we just need a marketing plan". Which of the five would you suggest first, and why?
2. Why does an operate mandate matter more than its share of today's revenue suggests?
3. Name one engagement that moved up the ladder and the finding that moved it.

MODULE 2

The market we grow in

> After this module you can quote the facts that explain why private healthcare in Nigeria needs a firm like ours now, name the forces that create demand for each thing we sell, and say which numbers we never cite.

Our clients run private facilities in a market where patients mostly pay from their own pockets, where costs have been repriced by two years of devaluation and inflation, where the doctors they need are leaving, and where capital is available but selective. Each of those forces creates a specific kind of demand for us, and a growth team that knows them can tell which prospects are in the market before the prospects say so.

71.9%

of current health spending in Nigeria was out of pocket in 2023, by the WHO and World Bank series[1]

21.7m

people enrolled in any health insurance scheme at the end of 2025, roughly one Nigerian in ten[3]

$685m

paid abroad for medical travel in 2025, up from $643m in 2024, by CBN balance of payments data[4]

~55,000

practising doctors in 2025, down from 66,241 a year earlier; 4,193 left in 2024[5]

The five forces, and what each one sells

FORCE	THE EVIDENCE	THE DEMAND IT CREATES FOR US
Patients pay cash	Out of pocket was 71.9% of current health expenditure in 2023 (WHO and World Bank)[1]. The National Health Accounts, on a different method, put it at 58.3% in 2024 against total health spending of ₦10.42 trillion[2]. Never mix the two series.	Pricing and tariff work, the patient's experience of the bill, front-desk conversion, collections. A cash hospital lives or dies on whether patients trust its price.
Costs were repriced	The naira averaged about 645 to the dollar in 2023, 1,480 in 2024 and 1,517 in 2025[10]. Inflation ran at 33.2% in 2024 and had eased to 15.39% by August 2026 on the rebased index[9]. In April 2024 BusinessDay estimated that about one in five small and medium private hospitals had closed under operating costs[7].	Turnarounds, cost-to-serve reviews, tariff resets, and owners who need someone to tell them the truth about whether the business still works.
The doctors are leaving	Practising doctors fell by about 17% in one year to roughly 55,000, and around two thirds of the 4,193 who left in 2024 went to the United Kingdom[5].	Recruitment, retention, leadership assessment, workforce planning and diaspora engagement. Retention has become a board issue.
Insurance is growing slowly	The NHIA Act 2022 made cover mandatory, and enrolment reached 21.73 million by the end of 2025, with Lagos second at 2.76 million[3].	HMO network strategy, tariff negotiation with payers, and claims recovery for hospitals waiting to be paid.
Capital is selective	Nigeria received about 8% of the $4.2 billion invested in African healthcare since 2016, and the IFC put $24.5 million into a Nigerian cancer facility in February 2026[6]. Evercare Lekki is owned by a $1 billion TPG Rise fund, and Iwosan has acquired Lagoon and Euracare[29].	Venture structuring, investor readiness, fundraising support, post-acquisition integration, and build and operate for owners with capital but no operator.

Medical travel is a domestic opportunity

Roughly $650 to $700 million a year leaves the country for treatment abroad[4], and a Health Federation of Nigeria estimate puts total Nigerian health spending at $34 billion in 2026[6]. Every "bring it home" thesis we have worked on (Clearview's fertility destination, Osteon's revision arthroplasty, the Lyfe Place medipark) rests on the same observation, which is that local care is often already better value and is not believed. That is a trust problem, and trust problems are what we sell against.

Quality starts low and moves when measured

In one SafeCare programme in Kaduna, 17 of 25 private facilities started at the lowest of five levels, and on reassessment 19 of them had moved up one or two levels[8]. The sample is small and the lesson is general: most private facilities have never been measured, and once they are, they improve. That is the case for the audit, and the case for accreditation readiness as an offer.

Where the facilities are

Lagos had 3,670 registered health facilities when HEFAMAA last published a count in 2020[11], and the agency continues to seal facilities that fail inspection. A current national count of private hospitals is not reliably published, so build target lists from named facilities rather than from a market-size number.

Numbers we never cite

"91.47% out of pocket", any "Africa healthcare market worth $259 billion by 2030" figure (no traceable source), "70 plus accredited HMOs" (no NHIA list behind it), "3.9 doctors per 10,000" until we have it from the source, and any 2022 or earlier naira price used as a 2026 anchor. The BusinessDay closure estimate is a press estimate and must be described as one, never as a survey. When a number is not in Appendix A, it does not go into anything we publish.

CHECK YOURSELF

1. An owner tells you insurance will save his hospital. What two numbers would you put in front of him?
2. Which force creates demand for the HR practice, and which creates demand for claims recovery?
3. Why do we say medical travel is a trust problem rather than a price problem?

MODULE 3

How organisations buy advice

> After this module you can explain, with evidence, why most of the people who will hire us are not looking today, why referrals fail when the referred firm is hard to understand, and what that means for how we spend our time.

Professional services are bought differently from products. The buyer cannot inspect the work before paying for it, so they buy the firm's reputation and the person in front of them. The research on how that happens is consistent, and it explains both our strengths and our gap.

Seven findings that shape this book

FINDING	THE EVIDENCE	WHAT WE DO ABOUT IT
Most buyers are not in the market	The Ehrenberg-Bass Institute estimates that only about 5% of business buyers are in market for a given category at any time[17].	A hospital owner changes adviser rarely. We have to be remembered in the years between purchases, so most of our effort goes into being known, not into chasing.
Growth needs both brand and pursuit	Binet and Field's analysis for the LinkedIn B2B Institute found business-to-business firms grow best with roughly 46% of effort on building the brand and 54% on activation[18].	Half the growth team's time goes on visibility (articles, rooms, Debo and the directors being seen), and half on named pursuit.
Referrals come from non-clients too	81.5% of professional services firms had received a referral from someone who had never been a client, and nearly half of those referrals were based on the firm's visible expertise[16].	Publishing what we know creates referrers we have never met.
Referrals die when the firm is unclear	51.9% of buyers ruled out a referred firm before speaking to it, and the top reason (43.6%) was that they could not understand how the firm could help them[16].	Every offer needs a page or a note a referred stranger can understand in two minutes. Module 18.
Clients refer when asked	In the same study 69% of clients were willing to refer, and 72% of those who had not referred said nobody had asked[16].	The referral ask is a formal step at the close of every engagement. Module 12.
Thought leadership gets you invited	86% of decision-makers said they were likely to invite a firm that produces high-quality thought leadership to bid, and 73% found it more trustworthy than marketing material[19].	Our diagnostic findings, written up properly, are the most persuasive material we have. Module 14.
Buyers research before they talk	61% of business buyers say they prefer a buying experience without a salesperson, and 73% actively avoid suppliers who send irrelevant outreach[20].	The website, the options note and the leave-behind do the early selling. Outreach is only sent when it says something specific about the recipient.

High-growth firms behave differently

Hinge's long-running study of professional services firms defines high growth as 20% a year or more for three years. In its consulting sample, high-growth firms were 82% more likely to research their target market regularly and 52% more likely to spend 15% or more of revenue on marketing[14]. The 2025 edition, covering 770 firms, found high-growth firms growing four times faster and 30% more profitable than their peers[15]. The

two habits that separate them are the two we have least of: systematic research on our own market, and a deliberate budget for being known.

Keeping a client is cheaper than finding one

The figure usually cited, from work by Bain's Fred Reichheld reported in Harvard Business Review, is that acquiring a new customer costs five to twenty-five times as much as keeping an existing one, and that a five-point rise in retention lifts profit by 25 to 95%[24]. The figures come from other industries and should be quoted as such, but our own record points the same way: the Medbury division, the Haven growth mandate and the House of Refuge retainer all began as smaller pieces of work for the same client.

> **What this means in one paragraph**
>
> Our network-led model already does the hard part, which is trust. What it cannot do is reach the 95% who are not yet looking, explain us to a stranger who has just been given our name, or remember to ask the client who would happily refer us. Those three gaps are what Parts II and III are built to close.

CHECK YOURSELF

1. Why would an inbound lead count make a good quarter look bad in this firm?
2. A director says "we do not need content, all our work comes from referrals". What does the Hinge referral study say to that?
3. What proportion of effort should go to being known, and what is the evidence?

MODULE 4

Who we serve

> After this module you can place any prospect in one of seven segments, name the event that usually puts them in the market, and say who else in their organisation influences the decision.

Organisations are split by what they own and what they are trying to do. Every prospect falls into exactly one row. Part IV has a module for each.

SEGMENT	WHO	WHAT PUTS THEM IN THE MARKET	USUAL FIRST PURCHASE	MODULE
Hospital owners and promoters	Doctor-owners and founders of single-site private hospitals and clinics	Cash pressure, an incident, a failed expansion, a HEFAMAA notice, a new site, a key resignation	Audit	19
Groups, boards and family businesses	Multi-site groups, family-owned hospitals, holding companies	Expansion, succession, a split board, business units the centre cannot direct	Governance or a scoped strategy piece	20
Investors, lenders and development finance	Funds, family offices, DFIs, banks, fintech lenders	A deal in diligence, a portfolio company in trouble, a new fund looking for an operator	Diligence, options note, structuring	21
Payers and HMOs	HMOs, corporate health schemes, takaful and mutual plans	Entering a new state, a provider network that is long but inadequate, rising medical loss	Network strategy	22
Government, foundations and federations	Ministries, state agencies, foundations, the HFN, professional bodies	A published tender, a programme with funding, a policy problem needing a brief	Tendered review or policy work	23
Specialists and clinics	Specialist practices, aesthetics, fertility, diagnostics, day-case centres	A new clinic, a referral pipeline that has dried up, a competitor opening nearby	Positioning or a front-desk programme	24
Workforce buyers	HR directors and medical directors of any of the above	A senior vacancy, a failed hire, staff turnover, a culture problem after an incident	Recruitment or leadership assessment	25

The buying group

The person who calls us is rarely the only person who decides. In a doctor-owned hospital the spouse, a sibling on the board and the accountant all have a view, and in a group the finance director and the business unit heads do. Assume every document we send will be forwarded to someone we have not met, and write it so that it works without us in the room. A proposal that only makes sense with Debo explaining it will lose to one that explains itself.

Profiles of the people we sell to most

The founder-physician, 55, Lagos. Built a forty-bed hospital over twenty years. Revenue flat since 2023, costs not. Trusts doctors more than consultants, and trusts people who have run a hospital more than either. Needs: someone who understands without being told, a finding he did not expect, and discretion.

The second-generation director, 38. Returned from abroad to a family hospital. Wants governance and a plan, and needs an outside voice the older generation will accept. Needs: a structure that settles who decides, and a document that can be shown to the family.

The group chief executive. Runs several business units with strong leads she will not override. Needs: options presented as decisions, respect for her managers' autonomy, and proposals split by who signs.

The investor or lender. Has capital and a thesis and no operator. Needs: an honest options note with ranges, what has to be true, and what kills the deal, plus someone who can run the asset afterwards.

CHECK YOURSELF

1. An HMO medical director asks about recruitment for its own clinic. Which segment, and which module?
2. Name three events that put a single-site owner in the market.
3. Who are the hidden members of the buying group in a family-owned hospital?

MODULE 5

Positioning and the message house

> After this module you can state our position in one sentence, back it with four pillars and their proof, and choose the right words and avoid the wrong ones.

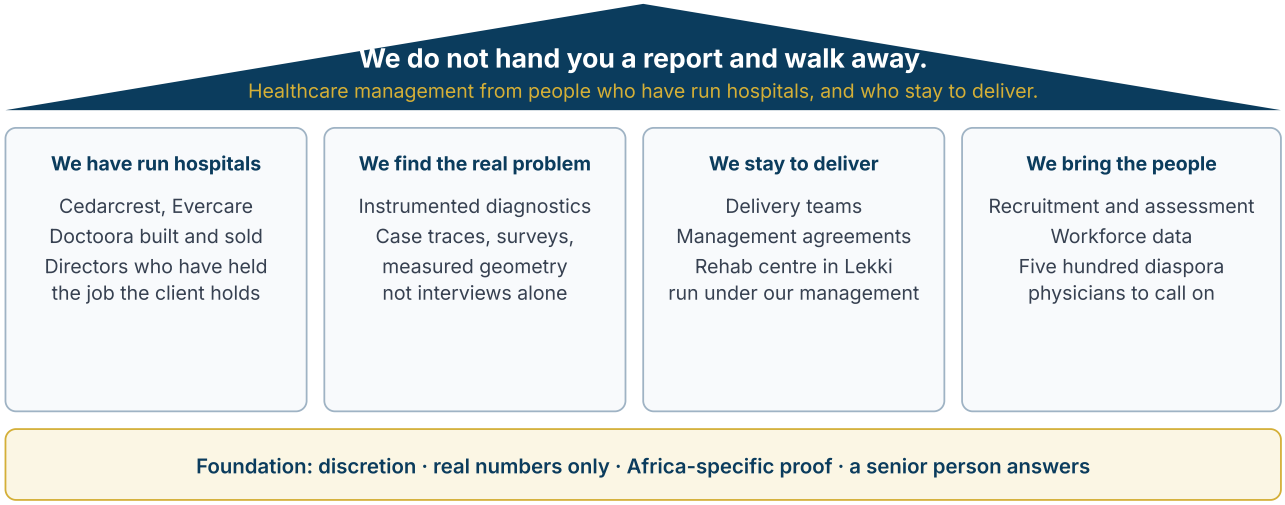


Figure 2. The message house. Every page, proposal opening and conversation draws on these four pillars.

How we sit against the alternatives

THE CLIENT'S ALTERNATIVE	WHAT THEY GET	WHAT WE SAY (NEVER NAMING THEM)
International strategy firms	A strong framework and a report, priced for multinationals	We have run hospitals in this market and we stay to deliver it
Local boutiques and individual consultants	The founder's time and interviews	A bench, instrumented diagnostics, and products that do the work after we leave
Software vendors	A system, with the outcome left to the client	We start from the outcome and use technology only where it helps
Recruitment agencies	A filled role	We assess whether the person can lead, and we live with the result inside a client we also advise
Doing nothing, or asking a friend	No fee and no change	A small, fixed piece of work that tells them what is actually wrong

Doing nothing is the competitor we lose to most often, and it is the one we should write for. An owner who has lived with a problem for three years needs a reason to act this quarter, which is why our best material names the cost of the status quo in the owner's own numbers.

Words

WE SAY	WE NEVER SAY	WHY
Turnaround, governance, tariff, receivables, occupancy, establishment	Synergy, disruptive, revolutionary, world-class solutions	Operators use the first list. The second reads as a pitch.
A structured leadership assessment	Maarova, CadreHealth, HospitIOS, in client body copy	We describe what is done, not which product does it

Intelligent analysis, structured review	AI-powered, AI-driven	Our audience hears a data privacy risk
We are writing to you	We are excited to invite you	Senior clinicians read the second as spam

CHECK YOURSELF

1. Write the one sentence and the proof point you would use with an investor.
2. Which pillar does the House of Refuge reference prove?
3. Why do we write against "doing nothing" rather than against other firms?

MODULE 6

The rules we never break

> After this module you know the rules that govern every message, page and proposal, and where the full version of each one lives.

The Marketing Handbook (section 3.5) has the full text. The short form is here because most of these rules exist because something went wrong once.

RULE	WHAT IT MEANS IN PRACTICE
No em dash	Anywhere: copy, headlines, bios, slides, interfaces. Use a comma, a full stop or a colon.
No AI framing	Describe the outcome, not the engine.
No platform names in client body copy	Name the products only in an appendix the client asked for. Product marketing is the exception.
No client named without written clearance	The only cleared reference is Pastor Tony Rapu's twenty-one bed rehabilitation centre in Lekki. Havana is never used as a reference.
No incident, patient outcome or regulatory finding in public	Ever. If unsure, the answer is no, and then ask Debo.
No price in marketing	Fees are discussed on a call. Staff documents carry the shape of a deal, never its fee or balance; figures live on /finance.
Check the relationship first	Before writing to anyone, ask how Debo knows them. Most of our prospects know him, and a cold register insults them.
Senior medic register	"Colleagues like you", "your name appears on". Never "job opportunities", "sign up" or "earn extra income".
Practitioners do not advertise themselves	Part F of the MDCN Code of Medical Ethics treats a doctor's self-advertisement as a disciplinary matter[26]. The facility advertises and the doctor educates.
Adverts are vetted before they run	Section 53 of the ARCON Act 2022 requires approval before any advert runs, including on social media, and health claims need supporting evidence[27]. Build the vetting time into every client campaign plan.
Consent before marketing	The NDPA 2023 gives everyone the right to object to direct marketing at any time, and 2025 decisions have treated unsolicited marketing messages without a lawful basis as unlawful[28]. Never buy a list, never reuse data collected for another purpose, honour every opt-out the day it arrives.
Bulk email goes through ZeptoMail, from hello@	Never Zoho for bulk. Verify the destination with a real payload before any send, and read the stored row back.
Be generous about people	Assume every internal document reaches the person it describes. Attribute shortfalls to the brief or the system, never to a named junior.

CHECK YOURSELF

1. A client's surgeon wants a billboard with his face and name on it. What do you advise?
2. A partner firm offers us a list of 2,000 hospital owners' phone numbers. What do you say?
3. Where does a fee figure live if a staff member needs it?

PART II

The growth system

Four engines bring work in, one pipeline keeps the record, and a small set of disciplines turns a conversation into a signed engagement and a signed engagement into the next one.

7. The growth model
8. The pipeline
9. Entry offers and the engagement ladder
10. Proposals, pricing and closing
11. Growing inside accounts
12. Referrals and references

MODULE 7

The growth model

> After this module you can name the four engines, say which one your own work feeds, and explain the single number that tells us whether the firm is becoming less dependent on one person.

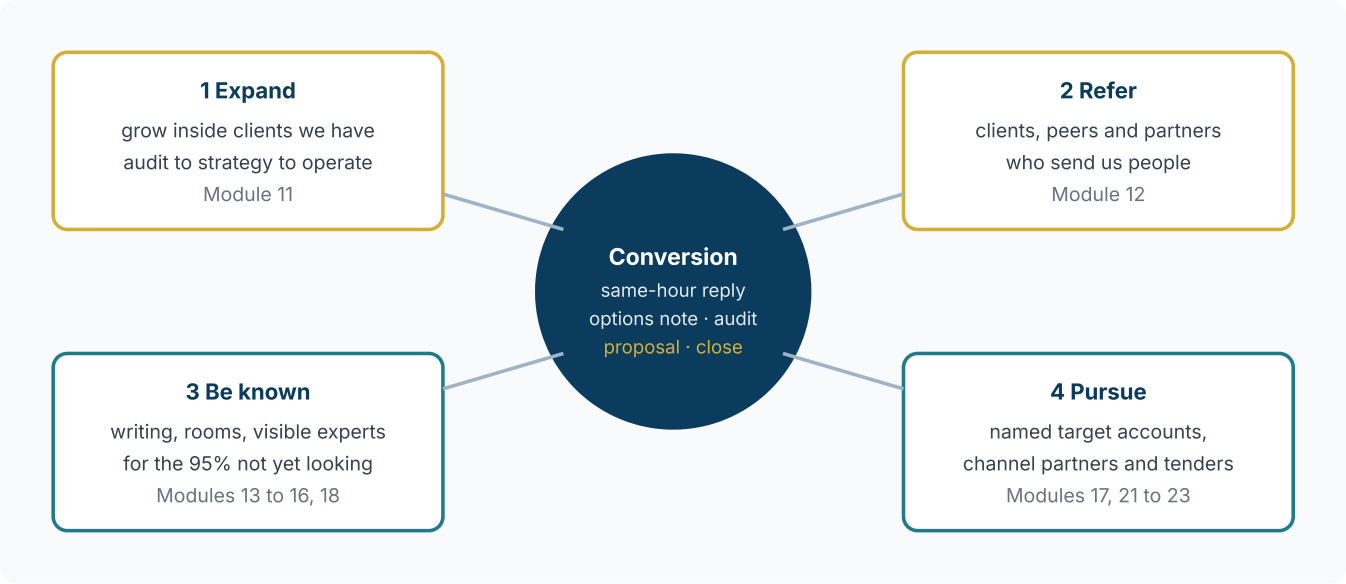


Figure 3. Four engines feed one conversion core. Gold engines run on relationships we already have; teal engines create relationships we do not have yet.

The two gold engines are where nearly all our work comes from today, and they depend on Debo's relationships and on the quality of our delivery. The two teal engines barely run. They are the ones that reach people Debo has not met, and building them is the main growth task for the next twelve months. None of this replaces the network. A visible firm makes Debo's introductions land faster, because the person he introduces us to can look us up and find something worth reading.

How effort should be split

The Binet and Field evidence suggests roughly half of effort on being known and half on pursuit[18]. For a firm of our size and stage we translate that as follows, measured in the growth team's hours rather than in money.

ENGINE	SHARE OF GROWTH EFFORT	WHO LEADS	WHAT GOOD LOOKS LIKE IN A QUARTER
Expand	25%	Engagement leads, with the BD lead	An account plan for every active client and a next-stage conversation opened before each engagement ends
Refer	15%	Debo and the BD lead	The referral and reference ask made at every close, and every referrer thanked and updated
Be known	40%	Marketing lead	Six articles, a monthly letter, one room, every director published at least once
Pursue	20%	BD lead	A worked list of fifty named accounts, partners actively referring, and every tender through the go or no-go gate

The number that matters most

We will track many numbers (Module 27), and one of them tells us whether this book is working: **the share of new qualified opportunities that did not come through Debo personally**. Today it is close to zero. The

proposal is 30% by the end of June 2027. It should rise without the absolute number of Debo's introductions falling, because the aim is to add engines, not to swap one for another.

DONE

- Expand engine proven: Haven, Medbury and House of Refuge all grew after the first piece of work
- Referral engine proven through Debo's network
- Lead, Referral, DiscoveryCall and Proposal models exist on the platform

PENDING

- No source field consistently recorded, so the headline number cannot yet be measured
- None of the ten case studies published, so the be-known engine has no fuel
- No target account list exists
- Agent channel and partner firm portal built, unused

CHECK YOURSELF

1. Which engine does a consultant on site at a client feed most directly?
2. Why does a visible firm make Debo's introductions convert faster?
3. What is the headline number and what is today's value?

MODULE 8

The pipeline

> After this module you can put an opportunity on the platform at the right stage, qualify it with the five questions, and apply the rules that stop a pipeline filling with people who were once polite to us.

A pipeline is only useful if it is the truth. Today the platform shows one live lead and four draft proposals dating from March and April, while the real pipeline (Duchess, Solace, Clearview, Iwosan, Afya, Grandville, the recruitment conversations and more) lives in Debo's head and in WhatsApp. Nobody can help with a pipeline they cannot see. The first rule of this module is therefore that **if it is not on /pipeline, it does not exist**.

Stages, and what moves an opportunity between them

STAGE ON THE PLATFORM	IT IS HERE WHEN	IT MOVES ON WHEN	OWNER
New	A name has arrived from any source, with the source recorded	Someone has looked at it within one working day	Office of the Founding Partner
Researching	We are finding out who they are, how Debo knows them, and what trigger may be in play	We know who signs and have a reason to talk	BD lead
Outreach	We have written or called with something specific	They reply	BD lead or Debo
Responded	They have replied and a conversation is possible	A discovery call is in the diary	BD lead
Discovery scheduled	A call or meeting is booked	The five questions below score seven or more	Whoever leads the call
Proposal sent	An options note or proposal has been sent, with a date to discuss it	Signed, or closed with a reason	Proposal owner
Converted or Lost	Signed and handed to delivery, or closed with a written reason	Lost opportunities go to nurture with a date to revisit	BD lead

The five questions

Score each from 0 to 2. Seven or more out of ten is a qualified opportunity, and only qualified opportunities count on the scoreboard.

QUESTION	0	1	2
Trigger: why now?	No reason to act this quarter	A general wish to improve	A named event: cash, incident, expansion, deal, deadline
Access: are we talking to the person who signs?	No	To someone who influences them	To the signatory, or they have asked us in
Fit: is this work we do well?	Outside the five things we sell	Adjacent	One of the five, in a setting we know
Means: can they pay at our rates?	No sign of it	Possibly, with a smaller first piece	Clearly, or they have named a budget
Next step: is there a date?	None	Vague	A meeting or decision date in the diary

The rules that keep it honest

- **Every record carries a source**: Debo introduction, client referral, non-client referral, partner, inbound web, inbound WhatsApp, event, content, target account outreach, tender. Without it Module 7's headline number cannot be measured.
- **Every record carries a next action and a date.** The Thursday sweep (Module 26) lists every opportunity with no logged action in fourteen days.
- **Forty-five days without a response** closes the opportunity as lost with a reason, or moves it to nurture with a date to revisit. Closing is not failure; an honest pipeline is the point.
- **A draft proposal older than thirty days** is sent, rewritten or closed. Six-month-old drafts make every forecast meaningless.
- **Inbound enquiries get a reply from a named person within one working hour.** In the HBR audit of 2,241 firms the average response took 42 hours and 23% never responded[21], so answering the same hour is the cheapest differentiator we have. The one website enquiry of the last month went unanswered for weeks because no inbox had an owner; that is a system gap, and the Office of the Founding Partner now owns it.

DONE

- Lead model with the stages above, qualification score and owner fields
- Thursday pipeline sweep run by the Office of the Founding Partner
- Contact and recruitment forms write Lead rows and notify hello@

PENDING

- Every live opportunity in Debo's head entered with stage, source, next action and date
- Decision on the four March and April drafts
- The whitepaper gate posts to a third-party form service and writes no Lead row; move it onto our own table
- No model for consulting opportunities larger than a Lead (founder decision whether one is needed)

CHECK YOURSELF

1. Score a prospect who met Debo at a wedding, owns a hospital, and said "we should talk sometime".
2. What happens to an opportunity with no logged action for fourteen days?
3. Why is "inbound lead volume" the wrong measure here?

MODULE 9

Entry offers and the engagement ladder

> After this module you can match a prospect to the right first offer, write the brief for an options note, and explain why our first offers are deliberately small.

A stranger will not sign a twelve-month mandate with a firm they met last week. They will buy something small, fixed and useful that lets them judge us, and our best engagements started exactly that way. The entry offers below are the first rung for each segment. Each one is designed to be worth its fee on its own and to show the client the next rung.

ENTRY OFFER	WHAT THE CLIENT GETS	BEST FOR	LEADS NATURALLY TO
Options note	A short note setting out the strategic options, capital and revenue as ranges, what has to be true, what kills each option, and what we cannot yet know	Investors, promoters, any prospect before a commitment	Diagnostic or feasibility
Organisational audit	A ten-domain diagnostic with live staff surveys, a case trace, a maturity assessment and a set of board decisions	Hospital owners, groups, boards	Turnaround, growth mandate, governance
Focused review	One domain done deeply: tariff and pricing, revenue cycle and receivables, workforce and establishment, clinical governance readiness	Owners with a named pain who will not yet buy a full audit	Audit or delivery in that domain
Board governance framework	Board and committee structure, delegation of authority, charters, a director induction	Family hospitals, groups taking outside capital	Board secretariat support, audit
Front-desk and client experience programme	A training and standards programme for reception and patient-facing teams, with a handbook	Clinics, aesthetics, fertility, diagnostics	Positioning, agency retainer, audit
Leadership assessment	Structured assessment of a leadership team or shortlisted candidates, with a report	HR and medical directors	Coaching, recruitment, culture work
Recruitment brief	A search for one named senior role, with assessment built in	Any employer with a senior vacancy	Further roles, workforce planning

The options note, in detail

The options note is the document we write most and the one that wins most. It is short (three to six pages), it uses ranges rather than point estimates, it states what has to be true and what would kill each option, and it is honest about what we do not yet know. It is deliberately not a feasibility study, because a confident twenty-six page plan answers every question and leaves the prospect no reason to call us. It carries no fee table. Its job is to make the prospect want the conversation in which the next piece of work is scoped.

The options note brief, which the BD lead fills before anyone writes

1. Who is it for, and how does Debo know them?
2. What is the trigger in their words?
3. What decision are they facing, and by when?
4. Which options are genuinely open, including doing nothing?
5. What do we know, what do we assume, and what can only a diagnostic tell us?
6. What is the next piece of work we would propose, and roughly what shape?

When the first piece is at our cost

We have carried audits at our own cost where the strategic value justified it (Osteon, Dennis Ashley). That is a founder decision each time, never a sales tactic, and it is never described to the client as a discount or a favour. Module 10 explains why.

DONE

- Options notes written for Solace, Kemi Balogun, Lyfe Place and others
- Audit method instrumented: live surveys, case trace, the link-style project page per client
- Front-desk programme delivered once and written up as a handbook

PENDING

- A one-page description of each entry offer that a referred stranger can read
- Focused reviews not yet packaged as named offers
- The front-desk programme not yet offered to a second clinic

CHECK YOURSELF

1. An investor asks what a forty-bed hospital in Abuja would earn. Which offer, and why not a feasibility study?
2. Which entry offer suits an aesthetics clinic whose enquiries do not convert?
3. Why does an options note carry no fee table?

MODULE 10

Proposals, pricing and closing

> After this module you can build a proposal that a forwarded reader will understand, apply the pricing rules, run the go or no-go gate on a tender, and know who may discuss a fee.

Who owns what

Debo owns price. The BD lead owns the shape of the deal: what is in scope, the stages, the timetable and the terms. Engagement leads own the delivery plan and the team. Nobody else discusses a fee with a client, and nobody discusses one before Debo has set the opening position.

How a proposal is built

- **Answer first.** The opening page states what we recommend and what it will achieve for the client, in their numbers. RAIN Group's research on what sales winners do differently found buyers rank demonstrated value above everything else[23].
- **Decisions, not recommendations.** Present options so the board is choosing, not being told.
- **Deck for the principal, prose for the file.** A busy principal gets a ten to fourteen slide landscape deck; the prose document is the leave-behind. Both must stand on their own, because buyers spend most of their decision time without us[20].
- **Split by who signs.** Where a group has strong business unit leads, send group-level and unit-level proposals to the people who own each decision.
- **No product names in the body.** Describe what is done. Name products only in an appendix the client asked for.
- **An opening partner proposal carries no fees.** When the relationship is a partnership, the first document sets out the model and principles; figures are discussed in a meeting.

How we price

Price has more leverage on profit than volume. In Marn and Rosiello's classic analysis, a 1% price improvement with no loss of volume lifted operating profit by 11.1%, against 3.3% for a 1% rise in volume[25]. Holding our price on the work we win matters more than winning one more engagement at a discount.

RULE	IN PRACTICE
Bill consultancy time	Setup and delivery work is priced as grades times days times published rates, with one visible programme concession. The rate card lives at /finance/rate-card.
Standard, one concession, net	The proposal shows the full standard fee, a single named concession and the net figure. It reads as transparent and it records what we gave.
Anchor, do not open at the floor	The opening position sits well above the walk-away. Concessions are held in a private ladder and spent one at a time, late, and only for something in return.
When pushed, cut scope, not rate	A lower fee buys less work. A lower rate teaches the client that our rates are negotiable.
Do not sell against ourselves	Never volunteer our margin, our costs, that we are absorbing anything, or a cap. State terms; do not argue them.
Invoices say non-refundable	Every invoice states its terms: non-refundable, free substitution of a named participant, one reschedule.

Tenders: the go or no-go gate

Open tenders are won less often than relationship work. Loopio's 2026 benchmark of more than 1,500 teams found an average win rate of 39%, and 81% of top performers use a formal go or no-go process[22]. Before any tender, the BD lead answers five questions in writing and Debo decides: do we know the buyer; do we have a reference they will accept; does the scoring favour us; can we staff it without taking partners off paying work; and is the fee worth the bid cost. Two noes means no bid.

Closing

A proposal is sent with a date to discuss it, never into silence. After the meeting, follow up within one working day with a written summary of what was agreed and what remains open. If a decision slips, ask what has changed rather than offering a discount, because a discount offered to a silent client answers a question nobody asked.

CHECK YOURSELF

1. A client says your fee is too high. Write your first sentence back.
2. Why is cutting scope better than cutting rate?
3. Run the five tender questions on a ministry RFP we heard about yesterday.

MODULE 11

Growing inside accounts

> After this module you can write a one-page account plan, spot the signals that a client needs the next piece of work, and run a value review with a client principal.

Every engagement we have that grew did so because somebody on our team noticed something the client needed and said so at the right moment. At Haven an audit about stock found a culture problem, and the culture work found a growth opportunity. At Medbury a location search led to a joint venture and then to running a division. This engine is the cheapest we have[24], and it runs on the people who are on site rather than on the growth team.

The account plan

Every active client has a one-page plan, written by the engagement lead in the first fortnight and reviewed monthly with the BD lead.

SECTION	WHAT GOES IN IT
The outcome	The client's own measure of success for this engagement, agreed in writing at the start
What we have noticed	Problems and opportunities outside our current scope, logged as they are seen
The next logical piece	The engagement that should follow this one, and the finding that will justify it
Who knows us	Everyone in the client who has seen our work, beyond the principal
Proof status	Whether a reference, a case study or a referral has been asked for, and the answer
Risk	Anything that could end the relationship: unpaid invoices, a sponsor leaving, a conflict

Signals worth logging

A senior vacancy or a resignation; a new site or service line under discussion; receivables the client mentions in passing; staff complaints about a manager; a board disagreement; an investor or lender in the conversation; a regulator's visit; a competitor opening nearby. Each one maps to something in Module 1 or Module 25. The consultant who notices it logs it on the account plan the same week and tells the engagement lead; the engagement lead decides when to raise it.

The value review

At the midpoint and the end of every engagement, the engagement lead meets the principal for thirty minutes with one page: the outcome agreed, what has happened against it, what we have learned about the organisation, and what we think should happen next. It is not a sales meeting, and it is the meeting in which most of our follow-on work has been agreed.

Cross-selling the products without naming them

When an audit finds weak leadership, propose "a structured assessment of the leadership team". When it finds unpaid HMO claims, propose "a receivables recovery programme". When a client needs a specialist, propose "a search, including senior Nigerians working abroad where the role suits a returner". The products do the work; the proposal describes it.

DONE

- Expansion has happened at Haven, Medbury and House of Refuge
- ClientExpansionRequest and ClientRecommendation models exist on the platform

PENDING

- No written account plan for any client
- Several active engagements (the Medbury division, House of Refuge strands, Dennis Ashley) have no Engagement or Client record, so they are invisible to planning
- Twenty-six completed leadership reports never followed by a coaching conversation

CHECK YOURSELF

1. You hear the finance manager say two HMOs owe the hospital six months. What do you do this week?
2. Who writes the account plan, and when?
3. Why is the value review not a sales meeting?

MODULE 12

Referrals and references

> After this module you can name our referral sources, make the ask at the right moment and in the right words, look after a referrer, and run the clearance process for a named reference.

Referrals are our largest source of work and we have never asked for one systematically. The research says most willing referrers simply have not been asked[16]. The ask is now part of closing every engagement, owned by the engagement lead, with the BD lead tracking it.

Who refers us

SOURCE	EXAMPLES	HOW WE LOOK AFTER THEM
Clients	Principals and directors of current and past clients	The ask at close; a note when something we did for them pays off
Peers who have not hired us	Doctors, owners and executives who know Debo or a director, board colleagues	The monthly letter; an invitation to a room; a short thank-you and an update whenever they send someone
Professional bodies	Health Federation of Nigeria, the Guild of Medical Directors, specialty associations, the diaspora physician network	Contributing policy work, speaking, hosting; never asking for business in their rooms
Advisers to owners	Lawyers, accountants, bankers, lenders, insurers who see a hospital in trouble before anyone else	A short briefing on what we do and when to call us; a reciprocal relationship
Commercial partners	Funding partners, equipment and implant suppliers, HMOs, technology partners	A named partner contact, a written referral route, and clarity on any fee

Making the ask

The right moment is when the client has just seen the value: the end-of-engagement value review, or the day a result lands. The engagement lead asks three things in one conversation, in this order. Would they be willing to be a named reference for a prospect who asks, by phone and only with notice? Is there someone they know who is facing a similar question? Would they allow an anonymised write-up of the method, which they will see before anyone else?

A good form of words: "We are a young firm and almost all our work comes through people who know us. If someone you respect is facing what you faced last year, I would be grateful if you would mention us, and I will make sure they are looked after."

Looking after a referrer

- Thank them within one working day, personally, from whoever they know best.
- Tell them when the conversation happens and, with the prospect's permission, how it went.
- Never pay a clinician for referring a patient. Fees exist only for business introductions through the agent channel or a written partner agreement.

References

We have one cleared named reference, the rehabilitation centre in Lekki run under our management. Getting to five by June 2027 is the highest-value proof task in the firm. Every reference needs written clearance

from the client principal, covering exactly what may be said and to whom; the clearance is filed with the client record. Havana is never a reference because Debo sits on its board.

DONE

- One named reference cleared and already used in our own advertising
- Referral model on the platform (client, consultant and staff referrals)
- Agent channel built for business introductions

PENDING

- The ask has never been made formally of any client; Haven is the natural first, at the end of its growth phase
- No referrer list or thank-you routine
- Agent channel has four approved agents and no deals

CHECK YOURSELF

1. When in an engagement do you make the ask, and who makes it?
2. A bank relationship manager sends us a hospital owner. What do you do in the next twenty-four hours?
3. Why is Havana never a reference?

PART III

Creating demand

How a firm becomes known to the people who will need it in two years, and how it reaches the people who need it now. Six modules on writing, speaking, convening, targeting and the website.

13. Visible expertise
14. The content engine
15. LinkedIn, email and WhatsApp
16. Rooms: roundtables, platforms and boards
17. Target accounts and outreach
18. The website as the place a referral lands

MODULE 13

Visible expertise

> After this module you can explain why named experts win work for professional firms, name the subject each director should be known for, and run the thirty-minute interview that turns a director's knowledge into a published piece.

People hire people. Nearly half of the non-client referrals in the Hinge study were based on the firm's visible expertise[16], and decision-makers in the Edelman and LinkedIn research said good thought leadership made them more likely to invite a firm to bid[19]. In this market the person is Debo, whose record is the firm's strongest single proof. The aim is not to make him less visible but to add four or five more named voices, so that a prospect who has never met him still meets someone senior who clearly knows the subject.

VOICE	KNOWN FOR	FIRST PIECES TO WRITE
Dr Debo Odulana	Running and turning around private hospitals; health venture structuring; the diaspora	What the audit finds that the owner did not ask about; pricing a hospital after the naira; why medical travel is a trust problem
Dr Adediwura Okeleye	Quality, accreditation readiness and clinical governance	What moving up an accreditation level actually takes; the clinical governance questions a board should ask
Oludayo Olugbamigbe	Workforce, recruitment and HR practice in health	The real cost of a failed senior hire; retaining doctors when the exit is open
Tito Ipinmoye	Product, strategy and running an audit	How an instrumented diagnostic differs from an interview round
Matthew Aloba	Data, compliance and evidence	The five numbers every private hospital should know by Monday

The thirty-minute interview

Directors are busy and most do not want to write. The marketing team interviews them for thirty minutes on one question, drafts the piece in their voice within three working days, and sends it back for one round of correction. Accuracy comes first, then clearance, then craft. A director should never spend more than an hour on a piece in total.

Debo's record, used properly

The person proof is publishable and underused: the former chief executive of Cedarcrest Hospitals Abuja, former Chief Innovation and Strategy Officer at Evercare Hospital Lekki, the founder who built Doctoora to twenty-one states and sold it to Evercare, and President of the Doctors Foundation for Care. In formal contexts he is always Dr Debo Odulana.

CHECK YOURSELF

1. Why add more voices rather than more Debo?
2. Which director would you interview for a piece on HEFAMAA readiness?
3. What is the order of review for a director's piece?

MODULE 14

The content engine

> After this module you can name the five formats we publish, the cadence for each, the one flagship piece of research we will run every year, and how one piece of work becomes five assets.

Our material is unusually strong and almost none of it is published. We have traced surgical cases end to end to find where money leaked, recovered a building's true geometry from a CAD file, and told a board its revenue record was a nursing handover sheet. Ten case studies are written and anonymised in the Marketing Handbook, and none is live. Most firms in this market publish generic commentary because they have nothing like this. The content engine exists to get our method into the world.

FORMAT	WHAT IT IS	CADENCE	OWNER
Insights article	800 to 1,500 words on one finding or method, on the website	One a fortnight	Marketing lead, with a director
Case study	The brief, what we found, the method, what it changed, the transferable principle, anonymised to the handbook standard	One a month until the ten are live	Marketing lead, cleared by Debo
The monthly letter	An email from Debo to the people who know us: one observation about the market, one piece we published, one thing we are watching	Monthly, first Tuesday	Marketing, in Debo's voice
Market brief	A two-page note on one force from Module 2, with sources: tariffs after devaluation, NHIA payments, the doctor exit	Quarterly	Marketing, with Matthew Aloba
The annual study	Original research: a survey of private hospital owners and directors on costs, prices, staffing and plans	Once a year, published in the first quarter	BD lead and marketing

The annual study

Hinge found high-growth consulting firms 82% more likely to research their target market regularly[14]. An annual survey of private hospital owners is the single piece of content most likely to make us the firm people quote. It gives every director something to talk about for a year, it is a reason to contact every target account, and its respondents become a list of people who have already engaged with us. The first edition should aim for at least eighty respondents, collected through Debo's network, the professional bodies and our own clients, with the instrument built in the same live-survey form we use in audits. Aggregate results only; nobody is named.

One piece, five uses

Every case study becomes an insights article (indexed and linkable), three LinkedIn posts from the person who led the work, a paragraph in the monthly letter, a slide in the firm profile deck, and an appendix page a proposal can carry. Write once, publish five times. The article is always first, because an article can be linked from a post, sent to a prospect and found by search, and a post cannot.

The first six to publish

From the handbook library, lowest sensitivity first: the building that was two hundred square metres bigger than the brief; the phase three decision taken in phase one; counting providers when the number that mattered was coverage; then the surgeon who blamed his postcode, once Dr Bola has seen it; then two method pieces (the ten-case trace and the live staff survey). The nursing handover sheet and the split board are high sensitivity and wait for Debo.

DONE

- Ten case studies written and anonymised with publication guidance
- Insights section live on the website, served from the Sanity studio
- Live survey forms built and proven in audits

PENDING

- No case study published
- No CFA newsletter or letter list exists (only CadreHealth has one)
- Annual study not designed
- No case study route on the site

CHECK YOURSELF

1. Why does the article come before the LinkedIn post?
2. Who must see a case study before it is published?
3. Name three ways the annual study creates pipeline.

MODULE 15

LinkedIn, email and WhatsApp

> After this module you can say what each channel is for in our business, the cadence for each, and the rules that keep us lawful and welcome.

Nigeria has about 13 million LinkedIn members, around 12% of internet users and growing by 18% a year[12], and it is where owners, directors and investors look a firm up. WhatsApp is used by over 96% of internet users[13] and is where business conversations actually happen once people know each other. Email remains the channel of record for proposals, letters and anything a client may forward.

CHANNEL	WHAT IT IS FOR	CADENCE	RULES
Debo's LinkedIn	The firm's most-read voice. Observations, findings, milestones, people	Two to three posts a week	Drafted by marketing, approved by Debo, never a hard sell
Directors' LinkedIn	Each director's subject (Module 13)	One post a fortnight each	Their own voice; marketing drafts if asked
Company page	Articles, case studies, hiring, the firm's news	Four a week	Every post links somewhere useful
Monthly letter (email)	Staying remembered by people who already know us	Monthly	Opt-in list only, sent from hello@ through ZeptoMail, one-click unsubscribe
WhatsApp	Conversations, follow-ups, forwarding a piece to someone who asked	As needed	Never cold. Never broadcast to people who have not agreed to hear from us.

Building the letter list lawfully

The letter goes only to people who have agreed to receive it. Start with Debo's direct contacts, each sent one personal note asking whether they would like the letter, and add the people who sign up on the website, at our rooms and through the annual study. Every address carries its source and the date of consent. Under the NDPA a person may object at any time and marketing must stop[28], so unsubscribes are honoured the same day and recorded as suppressions.

Before any bulk send

Send through ZeptoMail from hello@, never through Zoho. Post a real test payload to the live endpoint and read the stored row back. Confirm suppressions are loaded. For CadreHealth lists, the historic bounce back-fill must be complete first, because 11.09% of past sends hard-bounced and a second bad send damages the domain for every brand.

CHECK YOURSELF

1. Why is WhatsApp never used for cold contact?
2. Where does a new contact's consent date get recorded?
3. What must be true before a CadreHealth bulk send?

MODULE 16

Rooms: roundtables, platforms and boards

> After this module you can plan a closed-door roundtable from invitation to follow-up, choose which external platforms to accept, and explain why board seats and policy work are part of growth.

Hospital owners trust people they have sat in a room with. A room puts us in front of eight to twelve owners for two hours, lets them see how we think, and lets them meet each other through us, which is a service in itself. It is the most efficient trust-building activity we can run.

The roundtable

ELEMENT	STANDARD
Size and guests	Eight to twelve owners or directors, invited personally, one per organisation, no competitors seated together without their agreement
Subject	One live problem drawn from Module 2: pricing after the naira, retaining doctors, getting paid by HMOs, raising capital, succession
Format	Breakfast or early evening, two hours. A ten-minute opening from a director with one piece of data, then discussion under the Chatham House rule
Cadence	Quarterly in Lagos, twice a year in Abuja
Follow-up	A two-page note of themes (nobody named) to every guest within a week, an invitation to the letter, and a personal call to anyone who raised a problem we can help with
Measure	Opportunities opened within ninety days, not attendance

External platforms

Accept speaking invitations where the audience includes owners, investors or payers, and decline those where it is mainly vendors. Prefer panels where a director can share a specific finding over keynotes about the future of healthcare. Every talk ends with one piece people can take away and an invitation to the letter. The Health Federation of Nigeria, the Guild of Medical Directors, specialty associations and investor forums are the priority; check dates each quarter rather than relying on last year's calendar.

Boards and policy

Board seats and policy work (Debo's independent directorship at Havana, the housemanship brief drafted for the HFN) build standing that no campaign can buy. They are never used to sell. The rule is that a board seat creates a conflict to manage before it creates an opportunity to pursue, and any overlap between a board role and a commercial conversation is declared openly.

Events we run for clients are not our marketing

The Medlyfe launch evening on 21 October is the client's event and the client's brand. It shows what we can do, and it may be described later as method with the client's permission, but nothing about CFA appears in it.

CHECK YOURSELF

1. What is the measure of a roundtable's success?
2. Two competing hospital owners both accept. What do you do?
3. Why do we never sell in a professional body's room?

MODULE 17

Target accounts and outreach

> After this module you can build and tier the target account list, run the trigger watch, and write outreach a senior person would welcome.

Most of the people we want to work with are not looking for us[17], and 73% of business buyers avoid suppliers who send irrelevant outreach[20]. So we do not run campaigns at lists. We choose a small number of organisations we would most like to work with, learn them properly, and contact them only when we have something specific to say.

The list of fifty

The BD lead builds and owns a list of fifty named organisations across the segments in Module 4, chosen by fit (we could do excellent work for them), means (they can pay) and access (someone we know can introduce us). It is reviewed quarterly with Debo, who marks the relationship he has with each.

TIER	HOW MANY	TREATMENT
Tier 1	Ten	One-to-one. A named owner in the firm, a written account brief, a personal introduction through Debo or a mutual contact, an invitation to a room, a tailored options note when a trigger appears
Tier 2	Forty	One-to-few. Grouped by segment, invited to rooms, sent the relevant market brief personally, watched for triggers
Everyone else	Unlimited	One-to-many. The letter, LinkedIn and the website

The trigger watch

A trigger is the event that moves an organisation from the 95% into the 5%. The Office of the Founding Partner keeps a weekly log of triggers for the list of fifty and for clients, taken from the press, HEFAMAA notices, LinkedIn job changes, DFI disclosures, new facility openings, fund announcements and what our consultants hear. The Thursday sweep turns each relevant trigger into an action: a note from Debo, an invitation, or an options note.

Writing outreach that is welcome

- Check the relationship first. If Debo knows them, the message comes from him in a warm register.
- Open with something specific about them: a site they opened, a move they made, a problem their segment is facing. Never open with us.
- Offer something useful with no strings: the market brief, a seat at a room, a finding relevant to them.
- Ask for twenty minutes, with a reason.
- Senior clinicians: follow the senior medic register in Module 6 to the letter.

The lawful basis question

Writing one personal message to a named executive at their work address, about their business, is very different from a bulk campaign to a purchased list. We do the first and never the second. A written legal view on cold business outreach under the NDPA (including the claims recovery outreach to 799 hospitals) is a pending founder decision, and no bulk cold send happens until we have it.

DONE

- Hospital model on the platform with pipeline stage, distress scores and contact logs
- SalesCampaign with daily caps, tracked links and unsubscribe

PENDING

- The list of fifty does not exist
- No trigger log
- Legal view on cold outreach (founder decision)

CHECK YOURSELF

1. Name four sources for the trigger watch.
2. Rewrite "We are a leading healthcare consultancy offering..." as a first line to a hospital that just opened a second site.
3. Why are there only ten Tier 1 accounts?

MODULE 18

The website as the place a referral lands

> After this module you can explain the website's real job, list what every offer page needs, and name the fixes that come first.

In our business the website rarely creates a client. It decides whether a referral turns into a conversation. Somebody has been told our name, they look us up on a phone between meetings, and in two minutes they either understand how we could help them or they move on. More than half of buyers in the Hinge study ruled out a referred firm before speaking to it, most often because they could not see how it would help[16], and buyers who prefer to research alone[20] judge us there first.

What every offer page needs

1. The problem in the owner's words, in one line.
2. What the client gets in the first thirty days.
3. One piece of proof: a case study, a method, or a named reference where cleared.
4. The senior person who would lead it, with a photograph.
5. One call to action that reaches a named person who replies within the working hour.

Say less and hit hard. Depth lives in articles and frequently asked questions, not on the offer page.

Search

Search volume for consulting is small, and the searches that matter are by owners trying to solve a problem: how to register with HEFAMAA, how to price hospital services, how to get paid by HMOs, how to recruit a medical director. Each of those deserves a plain, accurate explainer that ends with an offer. We do not promise rankings and we do not buy links.

Fixes that come first

FIX	WHY
A case study route and the first three published	Proof is the single biggest gap on the site
One page per entry offer from Module 9	So a referred stranger can see what to buy first
Resolve "135+ years" against "60+" in documents	Two claims about the same thing undermine both
Move the whitepaper gate onto our own Lead table	It currently sends contact details to a third-party service and creates no record
A letter sign-up on every article	Converts readers into people we can stay in touch with
Contact form notifications sent as hello@	So every enquiry is visible in the mailbox Debo checks

CHECK YOURSELF

1. What is the website's real job in a referral-led firm?
2. List the five things an offer page needs.
3. Why does the whitepaper gate need to move?

PART IV

The plays

One module for each kind of buyer: what puts them in the market, what to offer first, what proof they need, where to find them, how the work grows, and what to watch for. The last module covers the products and the practices as growth lines of their own.

19. Hospital owners and promoters
20. Groups, boards and family businesses
21. Investors, lenders and development finance
22. Payers and HMOs
23. Government, foundations and tenders
24. Specialists, clinics and the agency
25. The products as growth lines

MODULE 19

Hospital owners and promoters

> After this module you can run the play for a single-site private hospital from first trigger to management agreement.

This is our core segment and the one the market forces in Module 2 hit hardest. Most owners are physicians who built the hospital themselves, carry it personally, and have never had it measured. They trust people who have run a hospital, and they do not like being sold to.

ELEMENT	THE PLAY
Triggers	Cash pressure or a missed payroll; an incident; a key doctor or manager leaving; a failed or stalled expansion; a HEFAMAA notice or inspection; an HMO dispute; a new competitor nearby; an offer from a buyer
First offer	The organisational audit, or a focused review if the owner names one pain (pricing, receivables, staffing)
Proof they need	That we have run a hospital; one case study that sounds like their problem; a director they would respect; discretion
Where they are	Debo's network, the Guild of Medical Directors, the HFN, specialty associations, their banker and accountant, roundtables
How it grows	Audit, then turnaround or growth mandate, then a management agreement; alongside, recruitment, leadership assessment, receivables recovery and marketing
Watch for	An owner who wants validation rather than truth; family members with different views; unpaid invoices from a client already in distress (stage payments in advance)

The conversation that works

Ask about the hospital before the problem: how it started, what it does best, what has changed since 2023. Then ask what the owner would fix first with a free hand. Then say what we usually find in hospitals like theirs, which is rarely what owners expect, and offer the audit as the way to know. Do not diagnose in the first meeting, and do not leave without a date for the next one.

The reference play

Haven's growth mandate runs to January 2027. If it meets its targets, Haven is the reference that sells this segment better than any article: a hospital whose owner can say what changed. Plan the reference ask now, for the January value review.

CHECK YOURSELF

1. An owner says "my problem is my staff". What do you offer and why?
2. Why do we not diagnose in the first meeting?
3. How should a distressed client's fees be staged?

MODULE 20

Groups, boards and family businesses

> After this module you can sell to a group without going around its business unit leads, and help a family-owned hospital settle who decides.

Groups buy differently from owners. The chief executive may want our help and still refuse to impose it on strong business unit leads, which is exactly what happened at Medbury and why proposals there are split between the group and each unit. Family hospitals face a different version of the same issue: the founder, the second generation and outside capital each need a structure they can trust.

ELEMENT	THE PLAY
Triggers	A new site or division; a joint venture; succession; a board that cannot agree; outside investors arriving; a unit underperforming
First offer	Board governance framework, or a scoped strategy piece for one decision; for a group, a small mandate that proves us to the unit leads
Proof they need	Evidence we respect their managers; fair-process governance work; build and operate experience
How it grows	Governance, then a strategy office or embedded team, then building and running new divisions under management agreements
Watch for	Being seen to take sides in a family or between units; conflicts between board roles and commercial work; a sponsor who is not the decision-maker

The Medbury relationship shows the full ladder: a location search, a joint venture, a division we now help set up and run. The lesson is that we earned each step with the person who owned that decision, and never by asking the chief executive to override her managers.

CHECK YOURSELF

1. Why are group proposals split by who signs?
2. What is the first offer to a family hospital about to take outside capital?
3. Name one conflict to declare before pursuing a board-adjacent opportunity.

MODULE 21

Investors, lenders and development finance

> After this module you can explain what capital providers need from an operator-adviser, and how one relationship with a funder can bring us several hospitals.

Capital for Nigerian healthcare is available and selective[6]. Funds, family offices and lenders need three things we can provide: an honest view of an asset before they commit, a plan they can hold management to, and someone who can run the asset if management cannot. One funder relationship can therefore send us a portfolio of work, which makes this the highest-leverage segment for the pursue engine.

ELEMENT	THE PLAY
Triggers	A deal in diligence; a fund close; a portfolio company missing plan; a new thesis that needs an operator; an acquisition needing integration
First offer	An options note on a specific asset or thesis; operational diligence; a hundred-day plan after acquisition
Proof they need	Ranges and kill criteria rather than optimism; Debo's operating and exit record; financial models that survive scrutiny
How it grows	Diligence on one asset, then portfolio support, then build and operate, then becoming the funder's preferred operator-adviser
Watch for	Being asked to bless a deal; equity-for-fees arrangements that conflict with advisory independence (founder decision each time)

Funders as channel partners

A lender sees a distressed hospital before anyone else. The claims recovery partnership with Carbon is the first example of a funder whose interest is aligned with ours: hospitals that need receivables financed are often hospitals that need their revenue cycle fixed. Banks, fintech lenders and DFI programmes are all potential channel partners on the same logic, each with a written referral route.

CHECK YOURSELF

1. What three things does a capital provider need from us?
2. Why is a lender a referral source for turnaround work?
3. What is the risk in equity-for-fees, and who decides?

MODULE 22

Payers and HMOs

> After this module you can sell network strategy to an HMO and explain how payer work connects to the rest of the firm.

Insurance covers about one Nigerian in ten[3] and is growing under the NHIA Act. HMOs entering a new state need a provider network that covers their members, not just a long list of hospitals, and our Lagos network work for a takaful HMO expanding south is the model: data-led selection anchored to where members actually are.

ELEMENT	THE PLAY
Triggers	Entry into a new state; a large new employer group; rising medical loss; regulatory pressure on network adequacy
First offer	Network strategy for one state, with provider acquisition
Proof they need	Coverage analysis rather than provider counts; access to specialists; speed
How it grows	Network strategy, then provider quality programmes, then tariff and contracting work
Watch for	Never mix payer clients into hospital-side receivables material; say "payor" in claims recovery copy, and keep each client's information separate

Payer work also strengthens our hospital work: we understand how HMOs select and pay providers, which is exactly what a hospital waiting on claims needs to know. Keep the two sides of that knowledge separate in every document.

CHECK YOURSELF

1. Why is a provider count the wrong measure of a network?
2. Which product helped justify our fee on the HMO network engagement?
3. What must never appear in claims recovery copy?

MODULE 23

Government, foundations and tenders

> After this module you can decide whether a tender is worth bidding, and explain how policy work builds standing without selling.

Public and philanthropic work is won by formal process, scored on method, experience, personnel and price, and lost more often than relationship work[22]. It is worth doing selectively for three reasons: it pays, it brings references outside the private sector, and it puts our directors in front of the people who shape the market.

ELEMENT	THE PLAY
Triggers	A published tender or request for proposals; a funded programme; a policy question a federation or ministry needs answered
First offer	A tendered review or evaluation; a policy brief
Proof they need	Named personnel with credentials; method scored against their criteria; references they accept
How it grows	Repeat cycles with the same funder; framework agreements; workforce intelligence offered as a dataset
Watch for	Bidding on strengths only (no padded capabilities); naming references only with their consent; partner time lost to bids we should not have entered

The Tony Elumelu Foundation tender is the template: a named engagement director, a bid built on what we actually do well, and references cited only with consent. Every tender goes through the five-question gate in Module 10 before any work starts.

CHECK YOURSELF

1. Name the four usual scoring criteria and where we are strongest.
2. What do we never include in a bid to look larger?
3. How does a policy brief create growth without selling?

MODULE 24

Specialists, clinics and the agency

> After this module you can run the play for a specialist practice or clinic, and explain where the healthcare marketing agency fits and what it must not do.

Specialists and clinics (aesthetics, fertility, orthopaedics, diagnostics, day-case surgery) are many, quick to decide, and usually limited by referrals and conversion rather than by clinical quality. They are also bound by the rule that the doctor may not advertise himself[26], which means their growth must come through the facility's brand, through education and through referring doctors.

ELEMENT	THE PLAY
Triggers	A new clinic or service; enquiries that do not convert; a referral source drying up; a competitor opening nearby; a plan to attract patients from abroad
First offer	Front-desk and client experience programme; positioning; a referral engine
Proof they need	That we understand admissions and referrals rather than impressions; the front-desk programme handbook; the specialist positioning work
How it grows	Positioning, then a digital front door, then an agency retainer, then an audit of the practice as a business
Watch for	Conflicts between clinics competing for the same patients; practitioner self-advertising; ARCON vetting lead times

The agency

The agency is the one service line delivered wholly by the marketing team. Its promise, from the Operating Brief, is that most agencies sell hospitals impressions and we sell them admissions, referrals and the service lines that carry margin. Its standard shapes and service levels are in the Operating Brief. Two growth rules apply here. No agency quote goes out until the agency rate card is approved (founder decision). And every new agency prospect is checked against current clients for conflict before the first meeting: a group with an aesthetics arm competes directly with the Medlyfe aesthetics business we help build.

CHECK YOURSELF

1. Why must specialist growth run through the facility's brand?
2. What must happen before any agency quote is sent?
3. Which first offer suits a fertility clinic with plenty of enquiries and few bookings?

MODULE 25

The products as growth lines

> After this module you can say where each product and practice stands, the one move that would grow it most, and how each one feeds the consulting work.

The products exist because the same things break in every institution we enter: leadership, workforce, receivables and the economics of a practice. They grow in two ways: as businesses with their own users, and as the means by which consulting recommendations get delivered. In client documents they are described, never named.

LINE	WHERE IT STANDS	THE ONE MOVE	HOW IT FEEDS CONSULTING
HR and workforce practice	Live, led by Oludayo Olugbamigbe. Recruitment, assessment, coaching, culture, benchmarking, interim. Two recruitment conversations waiting since May	Answer the waiting conversations with the existing pricing playbook	Every audit finds a people problem; this practice fixes it
Maarova	Live. Twenty-six leadership reports completed, four coaches onboarded, no coaching yet delivered	Offer each organisation holding a report a debrief that proposes coaching	Leadership assessment inside audits and recruitment
CadreHealth	Live. Over ten thousand professional records with salary data; activation around six per cent; historic bounces not yet back-filled	Finish the bounce back-fill, then offer workforce intelligence to an institution	Salary and workforce data for benchmarking, recruitment sourcing, the annual study
Mezo Health	Live at mezohealth.com. Nearly a thousand doctors seeded and very few claimed; claim links expire between 11 and 27 October; doctors told about their place converted far better than those who were not	Tell the seeded doctors before the links expire, following the senior medic register	Specialists for managed facilities; practices for clients needing capacity
Claims recovery	Page, directory and desk live; 799 hospitals imported; advances off until the funding partner signs	Sign the pilot terms, then reach hospitals through warm routes while the legal view on cold outreach is obtained	Finds hospitals with cash-flow pain, the turnaround trigger
Hospital operating software	Used inside managed facilities; staff app built, not deployed	None for growth. It is an internal tool	Makes our management agreements run; never named to clients

The flywheel, for internal use only

Workforce data makes our thought leadership and sources our placements. Engagements surface leadership problems, which assessment and coaching address. Doctors who want a practice rather than a job are who Mezo is for, and facilities we manage are where they can practise. Hospitals waiting on claims are hospitals that need their revenue cycle fixed. We tell this story inside the firm and never as a pitch.

CHECK YOURSELF

1. An audit finds three weak heads of department. Which lines could help, and how do you describe them in the proposal?
2. What must happen before any CadreHealth bulk send?
3. Why is the Mezo claim deadline the most urgent move on this page?

PART V

Operating system

Who does what, the weekly rhythm, the numbers we watch, the first ninety days, and the decisions only Debo can make. Keep this part open on your desk.

26. Team, roles and cadence
27. The scoreboard
28. The first ninety days and the open decisions

MODULE 26

Team, roles and cadence

> After this module you know what you own in the growth system, who you hand to, and which meeting you bring it to.

WHO	OWNS IN THE GROWTH SYSTEM
Dr Debo Odulana, Founding Partner	Relationships and introductions; price and every concession; the firm's voice; clearance of case studies and references; founder decisions
Dorathy Michael, Director of BD and Growth	The pipeline and its truth; the shape of every deal; the list of fifty; tenders; growth of the products. Full time from January 2027, and practice lead for the HMO network engagement until then
Toluwanimi Olayeni (Nimi), Marketing lead	Being known: content, the letter, rooms, the website, the directors' voices, the annual study; the agency
Ejiro Alechenu, Marketing	Production: articles to publication, LinkedIn scheduling, the letter build, event logistics
Engagement leads (Dr Adediwura Okeleye, Tito Ipinmoye, Matthew Aloba, Oludayo Olugbamigbe and others)	Account plans, value reviews, the referral ask at close, signals from site, method notes for case studies, one published piece a quarter
Ezinne Orji, Office of the Founding Partner	The record: every opportunity on /pipeline with source, stage and next date; the inbound inbox to the one-hour standard; the Thursday sweep; the commitments register; the trigger log
Abigail Oladejo, Office of the Founding Partner	New clients and contacts onto the platform; references and clearances filed; the letter list's consent records
Every consultant	Noticing. Anything outside scope that a client needs is logged on the account plan the same week

Until January, the BD lead's pipeline duties are covered by the Office of the Founding Partner for the record and by Debo for the relationships. Nothing in this book should wait for January if it can be done by the people already here.

The rhythm

WHEN	WHAT	WHO	OUTPUT
Monday	Engagement updates	Engagement leads	Signals from site added to account plans
Wednesday	Platform upkeep	Abigail	New clients, contacts and clearances recorded
Thursday, 45 minutes	The growth sweep	Debo, BD lead, marketing lead, Ezinne	Every stale opportunity actioned or closed; triggers turned into actions; what was published; what is blocked
First Tuesday	The monthly letter goes out	Marketing	Letter sent; new subscribers recorded
First Thursday	Scoreboard review	Same as the sweep	The Module 27 numbers, with one decision taken on the weakest
Quarterly	Book review and list of fifty review	Debo and the growth team	This book revised; the list re-tiered; next quarter's rooms and briefs set

CHECK YOURSELF

1. Who answers an enquiry that arrives on the website at 3pm on a Friday?
2. Which meeting do you bring a trigger to?
3. What does an engagement lead owe the growth system each quarter?

MODULE 27

The scoreboard

> After this module you can name the numbers we watch, why each matters, and today's value where it exists.

The scoreboard is short on purpose. Inbound lead volume is not on it, because in a referral-led firm it rewards the wrong activity. The targets are proposals for Debo to approve as a set.

NUMBER	WHY IT MATTERS	TODAY	PROPOSED, 31 MAR 2027	PROPOSED, 30 JUN 2027
Qualified opportunities live on /pipeline	Whether there is a pipeline anyone can help with	1	15	25
Share of new qualified opportunities not introduced by Debo	Whether the firm is becoming more than one network	Near zero, not measured	15%	30%
Cleared named references	The proof a stranger trusts most	1	3	5
Case studies published	Whether the be-known engine has fuel	0 of 10	4	6
Clients asked for a referral at close	Whether the cheapest engine is running	Not recorded	100%	100%
Active clients with an account plan	Whether expansion is deliberate	0	All	All
Inbound enquiries answered within one working hour	Whether a referral that finds us is caught	Not measured	90%	95%
Monthly letter subscribers, consented	The audience we can reach between purchases	0	300	600
Proposal win rate	Whether the offer and the qualification are right	Not measured	Measured	Above 50% for relationship work
Days from first conversation to signature	Sales efficiency	Not measured	Measured	Falling
Share of revenue from clients who bought before	Whether expansion is working	Not measured	Measured	Rising

The marketing budget

The firm has no budget line for being known. Hinge found high-growth consulting firms far more likely to spend 15% or more of revenue on marketing[14], and B2B growth evidence favours a near-even split between brand and activation[18]. A budget is a founder decision. The proposal is to set one in three lines (people, rooms and production, and research) for the first half of 2027, reviewed against the scoreboard at the end of June.

CHECK YOURSELF

1. Why is inbound lead volume not on the scoreboard?
2. Which number tells us whether this book is working?
3. What three lines would the marketing budget have?

MODULE 28

The first ninety days and the open decisions

> After this module you know what happens between now and mid-January, who does it, and which decisions are waiting on Debo.

Days 1 to 30, to 12 November: make the record true and catch what we already have

- Every live opportunity entered on /pipeline with source, stage, next action and date (Ezinne, with Debo, by 24 October).
- Inbound inbox and WhatsApp owned to the one-hour standard (Ezinne, from this week).
- The four March and April drafts sent, rewritten or closed (Debo decides at the first sweep).
- Mezo seeded doctors told before their claim links expire (product team, before 27 October).
- The waiting recruitment conversations answered (HR practice).
- First case study published on a new case study route (marketing and engineering).
- Account plans written for every active client (engagement leads, by 7 November).

Days 31 to 60, to 12 December: start being known

- The monthly letter launched to a consented starting list built from Debo's direct contacts.
- Two more case studies and the first two director pieces published.
- First Lagos roundtable held, on pricing and costs after the naira.
- The list of fifty drafted and tiered with Debo; the trigger log running.
- One-page offer descriptions for every entry offer, on the website.
- Maarova report holders offered a debrief.

Days 61 to 90, to 11 January: prepare for the BD lead and the year

- The annual study designed and in the field, for publication in the first quarter.
- Haven reference ask planned for the January value review.
- First scoreboard review on real data; targets approved.
- Handover to Dorathy Michael on arrival: the pipeline, the list of fifty, the trigger log and this book.

Open decisions for Debo

DECISION	WHY IT BLOCKS GROWTH
Approve the scoreboard targets as a set	Nobody can be held to a number that is not agreed
Set a marketing budget for the first half of 2027	Rooms, the annual study and production need money
Approve the agency rate card	No agency quote can go out without it
Clear the first three case studies for publication	The be-known engine has no fuel until they are live
Agree which clients to ask for a named reference, and when	Proof is the scarcest asset we have
Commission a legal view on cold business outreach under the NDPA	Claims recovery outreach and any target-account email depend on it
Resolve "135+ years" against "60+"	Two claims about the same thing undermine both

Decide whether consulting needs an opportunity record larger than a Lead	Multi-stage deals currently have nowhere to live on the platform
Name the conflict rule for competing clients in one specialty	The agency and the clinic play cannot grow safely without it

CHECK YOURSELF

1. What must be true of /pipeline by 24 October?
2. Which three things happen before Dorathy Michael starts full time?
3. Pick one open decision and write the one-line case you would put to Debo.

APPENDIX A

Sources

Every figure in this book with a superscript number comes from the source below. Where we could only see a figure through a reputable secondary report, the entry says so. Check the source before quoting any figure externally, and never quote a figure that is not on this list.

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Gaps to close before the next edition

A current national count of private hospitals from the Health Facility Registry; a current HEFAMAA count; the NHIA list of accredited HMOs; the current number of Nigerian-trained doctors on the UK medical register; the CBN primary table for medical travel; the ARCON penalty schedule; any data on how Nigerian doctors and executives use email. The annual study (Module 14) should fill some of these with our own data.

APPENDIX B

Campaign calendar, October 2026 to June 2027

MONTH	THEME	MAIN PIECES	ROOM
October	Getting the record right	First case study; Mezo claim outreach	None
November	What the audit finds	Two case studies; Debo on the problem behind the problem; letter launches	None
December	The private hospital in 2027	Market brief on costs, prices and the naira; year-end letter	Lagos roundtable: pricing after the naira
January	Pricing and tariffs	Tariff explainer; director piece on cost-to-serve	None
February	Getting paid	HMO receivables brief; claims recovery story if the pilot is live	Abuja roundtable: getting paid by payers
March	The annual study	Study published; every target account contacted with it	Lagos launch breakfast for the study
April	Keeping your doctors	Workforce brief from aggregate salary data; the cost of a failed senior hire	None
May	Governance for family hospitals	Board governance piece; succession	Lagos roundtable: succession and outside capital
June	Quality you can show	Accreditation readiness piece; half-year letter	None; scoreboard and book review

APPENDIX C

Checklists

Before you write to anyone outside the firm

1. How does Debo know them?
2. What is specific to them in the first line?
3. Is there an em dash, an AI phrase or a product name in client body copy?
4. Is any client named without clearance?
5. Does it work if forwarded to someone we have not met?
6. Is it going from hello@ with Debo copied where it is a client?

Qualifying an opportunity

1. Trigger, access, fit, means, next step: score each 0 to 2.
2. Seven or more is qualified.
3. Source recorded.
4. Next action and date recorded.

Before you quote a price

1. Has Debo set the opening position?
2. Is it built from the rate card?
3. Standard, one concession, net?
4. Is the concession ladder written and private?
5. Do the invoice terms say non-refundable?

Closing an engagement

1. Value review held with the principal.
2. Next logical piece proposed or noted.
3. Reference asked; answer and scope filed.
4. Referral asked.
5. Case study permission asked.
6. Account plan updated and the client moved to nurture.

Before any bulk send

1. Every address has a source and a consent date.
2. Suppressions loaded; bounces back-filled.
3. ZeptoMail from hello@, never Zoho.
4. Real payload posted to the live endpoint and the stored row read back.
5. One-click unsubscribe works.

Before a tender

1. Do we know the buyer?
2. Have we a reference they will accept, with consent?
3. Does the scoring favour us?
4. Can we staff it without taking partners off paying work?
5. Is the fee worth the bid cost? Two noes means no bid.

APPENDIX D

Glossary

TERM	MEANING HERE
Account plan	The one-page plan for an active client: outcome, signals, next piece, who knows us, proof status, risk
Engine	One of the four ways work arrives: expand, refer, be known, pursue
Entry offer	A small, fixed first engagement designed to stand alone and show the next one
List of fifty	The named organisations we most want to work with, tiered one-to-one, one-to-few and one-to-many
Options note	A short pre-engagement note with ranges, kill criteria and honest unknowns, and no fee table
Qualified opportunity	An opportunity scoring seven or more on the five questions
Room	A closed-door roundtable or briefing we host for owners and directors
The sweep	The Thursday growth meeting that keeps the pipeline true
Trigger	An event that moves an organisation into the market for what we sell
Value review	The midpoint and closing meeting with a client principal on outcome and next steps
95:5	The finding that only about one in twenty business buyers is in the market at any time

Related documents: the Marketing Handbook and the Marketing Operating Brief (docs/marketing), the Growth Pack (docs/office), the rate card (/finance/rate-card), the brand guide (docs/brand-guide-cfa.pdf). This book is built by scripts/build-cfa-growth-book.py from docs/growth/cfa-growth-book.html.